

D21 — Endocrine Consult ★

Dr. N. Raj, Endocrinology | Endocrinology | 02/26/2026 14:00

Patient: Pemberton, Harold J. **MRN:** 847-29-3156 **DOB:** 04/12/1951 **Allergies:** Sulfa drugs (rash)

ENDOCRINOLOGY CONSULT NOTE

Patient: Harold J. Pemberton | MRN: 847-29-3156
Date: 02/26/2026 14:00 | Provider: Dr. N. Raj, Endocrinology
Reason for Consult: Diabetes management – AKI, steroid-induced hyperglycemia, oral agents held

CURRENT DIABETES STATUS:
Type 2 DM on metformin 1000 mg BID + glipizide 5 mg BID at home. Both held during ICU stay. Currently on insulin sliding scale only. Glucoses ranging 200-340 over the past 5 days. Steroid-induced hyperglycemia is the primary driver (prednisone currently 20 mg, tapering).

HbA1c from last outpatient visit (11/2025): 7.4%

RECOMMENDATIONS:

- Metformin: Do NOT restart at this time.
 - Current Cr 1.9, eGFR approximately 35. AKI has not resolved (baseline Cr 1.5).
 - Metformin is contraindicated with eGFR < 30 and should be used with caution at eGFR 30-45.
 - Given active AKI with Cr not yet at baseline, the risk of lactic acidosis is too high.
 - PLAN: Recheck Cr in 1 week as outpatient. If eGFR > 30 and Cr stable at or near baseline, may restart at REDUCED dose of 500 mg BID (not full home dose of 1000 mg BID). Reassess dose at follow-up.
- Glipizide: Restart at 5 mg PO daily (REDUCED from home dose of 5 mg BID).
 - AKI impairs renal clearance of glipizide and its active metabolites.
 - At current renal function, full BID dosing carries significant hypoglycemia risk.
 - Once Cr returns to baseline, may consider increasing back to BID dosing.
- Insulin sliding scale: Discontinue at discharge.
 - Glipizide + dietary management should be sufficient as steroids taper.
 - If fingerstick glucose consistently > 300 during steroid taper, patient should contact PCP for interim management.

4. Follow-up: Endocrinology in 2 weeks (during steroid taper) with BMP and fingerstick log.

Electronically signed by: Dr. N. Raj, MD

Date: 02/26/2026 14:30

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